

Setting and question

In 2014 the Affordable Care Act offered states the option of expanding Medicaid to adults under 138% of the federal poverty line, and opened the Marketplace for subsidised private insurance. Medicaid spending rose in expansion states with the new enrollment. We test whether Medicaid expansion displaced population-level spending, utilization or coverage by other payers in the same state.

Data and design

IHME Disease Expenditure Project (DEX 2.0): state-level spending and encounters by payer and setting, all ages, 2010–2018, fit to SHEA state expenditure. Uninsurance from CPS. 25 states expanded on 1 January 2014; the 19 that never expanded serve as controls. Later expanding cohorts are dropped (n ≤ 3 per year). Callaway and Sant’Anna (2021) group-time difference-in-differences against the never-expanded states, conditioning on covariates X: poverty rate, median household income, pre-expansion uninsurance, and the shares under 18 and over 65:

ΔYt = Yt - Y2013

ATT(t) = E[ΔYt | D = 1] - E[E[ΔYt | X, D = 0] | D = 1]

Figures plot ATT(t); tables report the 2014–2018 average. Inference is a multiplier bootstrap, 1,000 draws, clustered on state. Parallel trends is a Wald test of ATT(t) = 0 across the three pre-expansion placebo years, χ²3 on the analytic clustered variance. Multiplicity is Benjamini–Hochberg across the 77 estimates in panel 4.

1. Expansion covered the uninsured

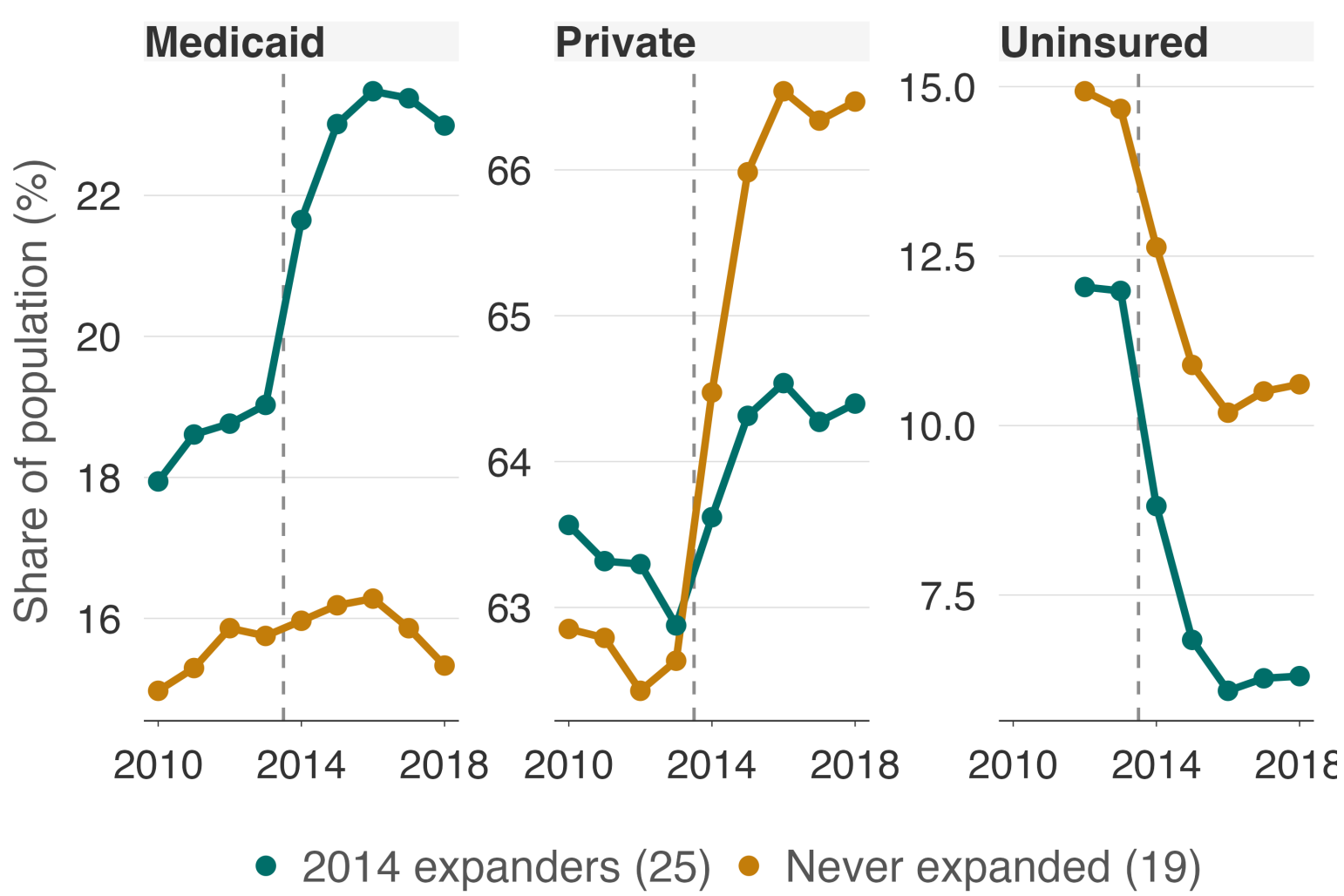


Table with 4 columns: Coverage, Effect, 95% CI, Pre-test. Rows include Medicaid, Medicare, Uninsured, and Private.

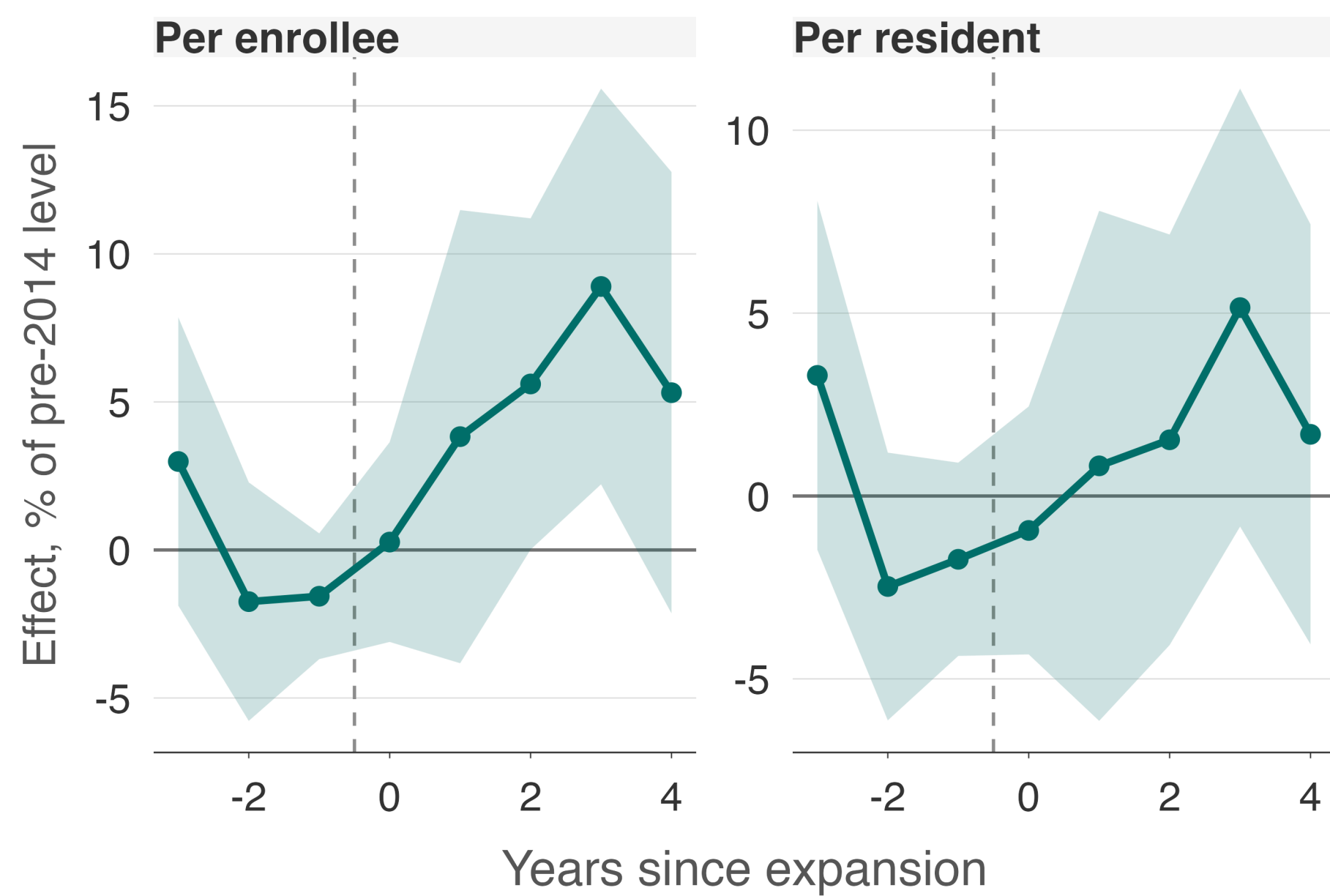
Medicaid coverage rose 3.7 points more in expansion states, and the uninsured rate fell a further 1.4 points. Private coverage grew in both groups, 62.9 to 64.4% in expanders and 62.6 to 66.5% in controls.

2. New Medicaid enrollees had similar healthcare consumption



Difference-in-differences effect at each year relative to 2014, as a percentage of the pre-expansion level in expansion states. 95% intervals from multiplier bootstrap. Spending across expansion states rose 18.0% (SE 3.9) after expansion alongside 19.9% increased enrollment, an additional \$189 per resident per year on a pre-expansion base of \$1,051. Spending per enrollee was not affected: -3.3% (SE 4.4).

3. No significant change in private spending



Private spending shows a small and non-significant increase in spend after expansion: +4.8% (SE 2.8) measured per enrollee and +1.7% (SE 2.4) per resident. These results do not show a compositional difference in new Marketplace enrollees between expansion and non-expansion states.

4. No effect outside Medicaid

We find no significant effect of the 2014 expansion on spending or encounters, per resident or per enrollee, in any setting, for any payer other than Medicaid. Null estimates are very precise for Medicare and for inpatient, emergency and ambulatory types of care. Imprecision is higher for out-of-pocket estimates and nursing and pharmacy care.

Spending per resident

Table with 5 columns: Setting, Medicaid, Medicare, Private, OOP. Rows include All care, Ambulatory, Emergency, Inpatient, Nursing, and Pharmacy.

Spending per enrollee

Table with 4 columns: Setting, Medicaid, Medicare, Private. Rows include All care, Ambulatory, Emergency, Inpatient, Nursing, and Pharmacy.

Encounters per resident

Table with 5 columns: Setting, Medicaid, Medicare, Private, OOP. Rows include Ambulatory, Emergency, Inpatient, Nursing, and Pharmacy.

Encounters per enrollee

Table with 4 columns: Setting, Medicaid, Medicare, Private. Rows include Ambulatory, Emergency, Inpatient, Nursing, and Pharmacy.

Effect as % of the pre-2014 level (SE). * p < 0.05 after adjustment for multiple comparisons.

Limitations

Estimates apply to the 2014 cohort and the never-expanded states through 2018. Impacts on late-expanding states, and long-term impacts after 2018 are not measured. Out-of-pocket reflects multiple types of payment. DEX combines cost-sharing by the insured with all spending by the uninsured, so compositional changes cannot be identified.

Population-level coverage and healthcare consumption data from DEX cannot identify individual switching behavior across insurance types.

Medicaid expansion did not produce strong changes in Medicare and private spending or utilization at the state level.

Medicaid spending per enrollee was unaffected by expansion. Private coverage increased in expansion and non-expansion states, but Marketplace enrollees did not have different consumption patterns than prior enrollees.